Sturse.

Module 6: Communication and Collaboration Study Guide

SECTION 01

Overview

This module, presented by Dr. Michelle Venn, introduces **TeamSTEPPS**, an evidence-based framework for optimizing team performance across the healthcare delivery system. It covers the four teachable-learnable skills (Communication, Team Leadership, Situation Monitoring, and Mutual Support), along with the tools used to apply them: closed-loop communication, SBAR, I-PASS, team event tools, situation monitoring checklists, mutual support strategies, and conflict and assertion methods. A patient handoff case study (Mr. Luis Ramirez) is used to identify communication and collaboration failures, and the module closes with diagnostic-safety data and the "Spirit of Inquiry" (Ask, Listen, Act).

SECTION 02

Key Points

- TeamSTEPPS rests on **four skills**: Communication, Team Leadership, Situation Monitoring, and Mutual Support.
- Safe and efficient care depends on the **coordinated activities of a multi-team system (MTS)**.
- **Closed-loop communication** confirms that critical information was sent, received, and verified.
- **SBAR** organizes urgent communication into Situation, Background, Assessment, and Recommendation.
- **I-PASS** structures handoffs and ends with a receiver synthesis.
- **Situation monitoring** (STEP and I'M SAFE) builds the individual awareness that produces shared mental models.
- **Mutual support** includes task assistance, formative feedback, advocacy and assertion, and the two-challenge rule.
- Many failures in the case study are **communication and handoff failures**, not clinical knowledge failures.
- Diagnostic errors and communication breakdowns occur across all care settings.

NURSING REMINDER

Nursing reminder: Safe care is a team outcome. Any team member, including the nurse, is responsible for speaking up when information is missing, unclear, or unsafe.

SECTION 03

Nursing Concepts & Clinical Implications

TeamSTEPPS Framework

The framework requires a **clearly defined and appropriate team structure** and uses four skills:

Communication

Team Leadership

Situation Monitoring

Mutual Support

The framework shows how these skills build:

Knowledge: Shared Mental Model

Attitudes: Mutual Trust, Team Orientation

Performance: Adaptability, Accuracy, Productivity, Efficiency, Safety, High Reliability

Sustainability: Enduring Culture of Safety

Together, these contribute to "highly reliable, safe, and effective care for every patient."

Communication Skill

Communication is a **verbal and nonverbal process** by which information is clearly and accurately exchanged among team members.

Barriers listed in the document include **varying communication styles**, **lack of information sharing**, **misinterpretation of cues**, and **distractions**.

Team Leadership

Team leadership is the ability to lead teams to maximize team member effectiveness by ensuring that:

> team actions are understood,
>
> changes in information are shared, and
>
> team members have the necessary resources.

Multi-Team System for Patient Care: Safe and efficient care involves coordinated activities across a multi-team system. In other settings, there may be **no single core team** and **multiple leaders** coordinating aspects of the patient's health and well-being. Some teams may be **entirely virtual** and include providers from **multiple organizations or care systems**.

Multi-Team System Components

The document lists these components of the multi-team system for patient care, in order:

1. Core Team, Including Patient
2. Contingency Teams
3. Coordinating Teams
4. Ancillary and Support Services
5. Administration

Situation Monitoring

Situation Monitoring (individual skill): continually scanning and assessing a situation to gain and maintain understanding of what is happening around you.

Situation Awareness (individual outcome): knowing what is going on regarding the patient, team members, the environment, and progress toward goals.

Shared Mental Models (team outcome): results when each team member maintains situation awareness and communicates so that all are "on the same page."

STEP Model

Situation monitoring is organized around four areas:

Status of the Patient: patient history, vital signs, medications, physical exam, plan of care, psychosocial issues, patient preferences or concerns

Team Members: fatigue, workload, task performance, skill, stress level

Environment: facility information, administrative information, human resources, triage acuity, equipment

Progress Toward Goal: status of the team's patient(s), established team goals, team tasks and actions, whether the plan is still appropriate

I'M SAFE Checklist

Used to determine the ability of you or your team members to perform safely:

I = Illness

M = Medication

S = Stress

A = Alcohol and Drugs

F = Fatigue

E = Eating and Elimination

Cross-Monitoring

A harm-error reduction strategy that involves:

monitoring the actions and stress levels of other team members,

providing a safety net within the team,

ensuring mistakes or oversights are caught quickly and easily.

The document describes this as "watching each other's back." A subacute care example is shown in the module but is not described in text.

STAR Self-Check

Each team member self-checks using four steps:

1. **STOP:** pause to focus on the immediate task.
2. **THINK:** think methodically and identify the correct action.
3. **ACT:** perform the action.
4. **REVIEW:** confirm the anticipated result occurred, or apply a contingency if required.

The document adds: "Think before you act!" and "FOCUS" on the intended action, the situation, and the expected outcome.

Closed-Loop Communication

A strategy used to communicate important or critical information. The loop has four parts:

1. The **sender** initiates the message.
2. The **receiver** accepts the message and provides feedback confirmation.
3. The **sender** verifies the message was received.
4. The loop is closed.

Forms of closed-loop communication:

Call-Outs: inform all team members simultaneously during situations, help team members anticipate next steps, and direct responsibility by name to the specific individual responsible for the task.

Check-Back: used to ensure information conveyed by the sender is correctly understood by the receiver.

Teach-Back: confirms the sender explained information clearly and that patients or family members understand. The sender asks the patient or family member to explain the information or actions needed, in their own words.

Handoff: a standardized method for transferring information, along with authority and responsibility, during transitions in care.

Closed-Loop Example (Dr. Moss and Mary)

Dr. Moss asks Mary to share a cholesterol management information pamphlet with Mr. Garcia and arrange a follow-up visit in a month.

Mary confirms, repeating the task back: she will share the pamphlet and arrange the follow-up visit for Mr. Garcia in a month.

Dr. Moss responds, "Correct," which closes the loop.

Teach-Back Process

If the patient teaches back correctly and there is more to explain: chunk and teach information, then ask the patient to teach back in their own words. Allow the patient to consult material.

If the patient does not teach back correctly: reteach using different words.

SBAR

SBAR is used for structured communication. The document's questions for each element are:

Situation: What is going on with the patient?

Background: What is the clinical background or context?

Assessment: What do I think the problem is?

Recommendation or Request: What would I do to correct it?

SBAR Example: Mr. Webb (Rural Community Clinic)

A nurse from a 5th Street office calls a physician about Mr. Webb:

Situation: He reports substantial discomfort and not much urine in his catheter bag.

Background: He is 83 years old and has a catheter in place during recovery from bladder cancer treatment.

Assessment: He reports a temperature of 100.4, and his urine is cloudy and slightly red. The nurse is concerned he may have an infection and that his catheter may be clogged.

Recommendation: The nurse requests that he come into the office this morning for the physician to see him, and asks whether the physician would like labs, including blood cultures, obtained on arrival.

The document presents this under "Using CUS and SBAR in a Rural Community Clinic."

I-PASS Handoff

The document presents the following structured handoff components, in order:

1. **Illness Severity:** stable, watcher, or unstable.
2. **Patient Summary:** includes a summary statement; events leading up to admission or care transition; hospital course or treatment plan; ongoing assessment; and contingency plan.
3. **Action List:** a to-do list with timelines and ownership.
4. **Situation Awareness & Contingency Planning:** know what is going on and plan for what might happen.
5. **Synthesis by Receiver:** the receiver summarizes what was heard, asks questions, and restates key actions or to-do items.

NURSING REMINDER

Nursing reminder: A handoff transfers information along with authority and responsibility. If the receiver cannot restate the key actions and to-do items, the handoff is not complete.

Team Event Tools

Three tools are used to structure team work:

Brief (sharing the plan): a short session before the start to share the plan, discuss team formation, assign roles and responsibilities, establish expectations and climate, and anticipate outcomes and likely contingencies.

Huddle (monitoring and modifying the plan): an ad hoc meeting to ensure continual progression of care toward the goal. Purposes include re-establishing or affirming situational awareness, reinforcing the plan in place, and assessing the need to augment or adjust the plan to optimize outcomes.

Debrief (reviewing team performance): a structured, intentional yet informal, quick information exchange designed to improve team performance through lessons learned and reinforcement of positive behaviors.

Brief Checklist

- Who is on the team?
- Do all members understand and agree upon goals?
- Are roles and responsibilities understood?
- What is our plan of care?
- What is staff and provider availability throughout the shift?
- How is workload shared among team members?
- What resources are available?
- What matters to you related to the focus of the briefing?

Debrief Checklist

- Was communication clear?
- Were roles and responsibilities understood?

- Was situation awareness maintained? (Did you know the plan?)
- Was workload distribution equitable?
- Was task assistance requested or offered?
- Were errors made or avoided?
- Were resources available?
- What went well?
- What one thing should improve?
- What is one thing that could be done differently next time?

Mutual Support

Task Assistance

Helping others with tasks builds a strong, trusting team. Key strategies:

- Team members foster psychological safety and protect each other from work overload.
- Effective teams place all offers and requests for assistance in the context of patient safety.
- Team members foster a climate where it is expected that assistance will be actively sought and offered.
- Resilient teams are willing to ask for help and take responsibility for facing challenges and finding solutions.
- Assistance is sought from and provided to patients and family caregivers.
- Team resilience is strengthened by asking, "I have **10 minutes**. How can I help?"

Formative Feedback

Feedback is information given to team members through verbal or nonverbal communication, intentionally or unintentionally. Formative feedback is shared to improve team performance and should be:

- **Appreciative:** expresses gratitude and notes actions done well.
- **Timely:** given soon after the target behavior occurs.
- **Respectful:** focuses on behaviors, not personal attributes.
- **Specific:** relates to a specific task or behavior that requires correction or improvement.
- **Directed toward Improvement:** provides directions for future improvement.
- **Considerate:** considers the team member's feelings and delivers negative information with fairness and respect.
- **Patient focused:** addresses the impact of team behaviors on the patient's well-being.

Advocacy and Assertion

- **Advocate for the patient.** This is invoked when team members' viewpoints do not coincide with those of the decision-maker.
- Assert a corrective action in a firm and respectful manner, following these steps in order:

1. Make an opening.
2. State the concern.
3. State the problem (real or perceived).
4. Offer a solution.
5. Reach agreement on next steps.

Two-Challenge Rule

Empowers all team members to "stop the line" if they sense or discover an essential safety breach. The document pairs this with three statements: **"I am CONCERNED!"**, **"I am UNCOMFORTABLE!"**, and **"This is a SAFETY ISSUE!"**, followed by **"Stop the Line."** The document lists CUS as a tool but does not expand the acronym.

DESC Conflict Approach

A constructive approach for managing and resolving conflict:

Describe the specific situation or behavior; provide concrete data.

Express how the situation makes you feel and what your concerns are.

Suggest other alternatives and seek agreement.

State **consequences** in terms of impact on the patient and established team goals; strive for consensus.

Case Study: Mr. Luis Ramirez

Mr. Luis Ramirez, age 72, is being transferred from the ED to a medical-surgical unit after treatment for heart failure exacerbation. Key facts from the case:

He speaks primarily Spanish. His adult daughter is bilingual but is not always available.

In the ED, a nurse tells the receiving nurse, "He is stable and probably going home tomorrow."

The chart contains an incomplete medication-reconciliation note.

He received IV furosemide and has a history of falls.

The ED provider verbally tells the hospitalist that the patient "seems confused," but this is not communicated during the nursing handoff.

The daughter later says her father does not understand his medication changes, lives alone, and has limited transportation.

The document asks students to:

1. Categorize each failure as related to one of the five categories below.
2. Identify at least four communication or collaboration failures.
3. Identify the possible patient-safety consequences of each failure.

The five categories are:

- Communication process
- Culture, language, health literacy, or inclusion
- Role clarity or hierarchy
- Handoff/transition coordination
- Escalation or safety culture

Failure Analysis Framework

Using the case facts, possible failures and categories include:

- **Confusion not included in nursing handoff** (ED provider told the hospitalist; nurse handoff omitted it). Categories: Handoff/transition coordination; Communication process.
- **Premature stability statement** ("stable and probably going home tomorrow") with an incomplete medication reconciliation. Categories: Communication process; Handoff/transition coordination.
- **Language barrier with the patient** and reliance on a daughter who is not always available. Category: Culture, language, health literacy, or inclusion.
- **Father does not understand medication changes** after discharge planning. Categories: Culture, language, health literacy, or inclusion; Handoff/transition coordination.
- **Concern about confusion not escalated** in a patient with IV furosemide and a history of falls. Categories: Escalation or safety culture; Role clarity or hierarchy (concern routed provider-to-provider rather than through the nursing team).

NURSING REMINDER

Nursing reminder: Patient-safety consequences should be reasoned from the case facts. For example, a change in mental status that is not communicated, in a patient with a fall history who received IV furosemide, creates a safety risk that the receiving team cannot plan for.

Diagnostic Safety Data

The document presents the following data on diagnostic errors and communication:

- 1 in 3 patients experiences a diagnostic error firsthand.
- **33%** of all diagnostic-related malpractice cases have one or more communication breakdowns.
- Of diagnostic-related malpractice cases: **4%** low severity, **27%** medium severity, **69%** high severity; **34%** resulted in death.
- Diagnostic-related communication failures occur across all settings: outpatient **55%**, emergency department **23%**, inpatient **22%**.
- Inappropriate testing, wrong treatments, and malpractice lawsuits result in expenses over $100 billion per year.

Spirit of Inquiry

Three steps guide diagnostic safety:

Ask: Questions are the path to discovery and convey value. How do I ask the right questions of the right people at the right time to achieve a safe diagnosis?

Listen: Questions are only meaningful if I listen actively through mindful engagement. How do I integrate what I hear with what I already know to ask what else it could be?

Act: Asking and listening are followed by thoughtful action and a plan that includes patient perspectives, leading to better health.

Barriers to Effective Teamwork

The document lists these barriers:

- Inconsistency in team membership
- Lack of time
- Lack of information sharing
- Hierarchy
- Defensiveness
- Conventional thinking
- Complacency
- Varying communication styles
- Conflict
- Lack of coordination and follow-up
- Distractions
- Fatigue and burnout
- Workload
- Misinterpretation of cues
- Lack of role clarity

Tools and Strategies

Communication: SBAR, Call-Out, Check-Back, Handoff, Teach-Back, I-PASS

Leading Teams: Brief, Huddle, Debrief

Situation Monitoring: STEP, I'M SAFE, Cross-Monitoring, STAR

Mutual Support: Task Assistance, Formative Feedback, Advocacy and Assertion, Two-Challenge Rule, CUS, DESC

Outcomes

The document lists these outcomes of effective teamwork:

- Shared Mental Model
- Adaptability
- Team Orientation
- Mutual Trust
- Reduced Burnout
- Psychological Safety
- Effective Team Performance
- Safe, Highly Reliable, Patient-Centered Care

The module references a "Labor and Delivery: Successful Outcomes Using TeamSTEPPS" example but does not describe it in the text.

SECTION 04

Core Definitions

TeamSTEPPS: an evidence-based framework to optimize team performance across the healthcare delivery system, built on four skills and requiring a clearly defined and appropriate team structure.

Communication: a verbal and nonverbal process by which information can be clearly and accurately exchanged among team members.

Team Leadership: the ability to lead teams to maximize the effectiveness of team members by ensuring team actions are understood, changes in information are shared, and team members have necessary resources.

Situation Monitoring: the process of actively scanning and assessing situational elements to gain information or understanding, or to maintain awareness to support team functioning.

Mutual Support: the ability to anticipate and support team members' needs through accurate knowledge about their responsibilities and workload.

Situation Awareness: an individual outcome; the state of knowing what is going on around you regarding the patient, other team members, the environment, and progress toward goals.

Shared Mental Model: a team outcome that results when each team member maintains situation awareness and communicates to ensure all members are "on the same page."

Multi-Team System (MTS): the coordinated activities of multiple teams that provide safe and efficient patient care.

Closed-Loop Communication: a strategy used to communicate important or critical information, consisting of sender initiation, receiver acceptance with feedback confirmation, and sender verification.

Call-Out: informs all team members simultaneously during situations, helps them anticipate next steps, and directs responsibility by name.

Check-Back: ensures the information conveyed by the sender is correctly understood by the receiver.

Teach-Back: confirms the sender explained information clearly and that the patient or family member understands, by asking them to explain it in their own words.

Handoff: a standardized method for transferring information, along with authority and responsibility, during transitions in patient care.

Brief: a short session before the start to share the plan, assign roles, establish expectations, and anticipate contingencies.

Huddle: an ad hoc meeting to ensure continual progression of care, re-establish or affirm situational awareness, and assess the need to adjust the plan.

Debrief: a structured, intentional yet informal, quick information exchange designed to improve team performance through lessons learned.

STEP: the four areas of situation monitoring: Status of the Patient, Team Members, Environment, and Progress Toward Goal.

I'M SAFE: a checklist for determining the ability of you or your team members to perform safely (Illness, Medication, Stress, Alcohol and Drugs, Fatigue, Eating and Elimination).

Cross-Monitoring: a harm-error reduction strategy involving monitoring the actions and stress levels of other team members and providing a safety net.

STAR: a self-check for each team member: STOP, THINK, ACT, REVIEW.

Task Assistance: helping others with tasks to build a strong, trusting team.

Formative Feedback: information shared to improve team performance; should be appreciative, timely, respectful, specific, directed toward improvement, considerate, and patient focused.

Advocacy and Assertion: advocating for the patient when viewpoints differ from the decision-maker, and asserting corrective action firmly and respectfully.

Two-Challenge Rule: empowers all team members to "stop the line" if they sense or discover an essential safety breach.

DESC: a constructive approach for managing and resolving conflict (Describe, Express, Suggest, Consequences).

SECTION 05

NCLEX-Style Focus Areas

Identifying the four TeamSTEPPS skills. Know that Communication, Team Leadership, Situation Monitoring, and Mutual Support are the four skills, and that they connect to Knowledge, Attitudes, Performance, and Sustainability.

Closed-loop questions. Expect scenarios asking which step is missing when the sender does not verify receipt, or which strategy (call-out, check-back, teach-back, handoff) fits a situation.

Teach-back priorities. If a patient does not teach back correctly, the nurse reteaches using different words. The nurse does not simply repeat the same explanation.

SBAR sequencing. Know the order (Situation, Background, Assessment, Recommendation) and the question each element answers. Know that SBAR is used in the rural clinic example to communicate a possible infection and clogged catheter.

I-PASS components. Expect questions about illness severity categories (stable, watcher, unstable), the patient summary, the action list, contingency planning, and the receiver's synthesis.

Handoff failures. Scenarios may ask which failure occurred when a change in mental status is reported to one provider but not included in the nursing handoff, or when the receiver cannot restate key actions.

Situation monitoring. Know the four STEP areas and the six I'M SAFE items. Expect questions where a nurse notices fatigue or stress in a colleague and should apply cross-monitoring.

Language and health literacy. Recognize that reliance on an intermittently available bilingual family member, and a family member's misunderstanding of medication changes, are culture, language, or health literacy issues.

Escalation. Know the two-challenge rule and the "stop the line" language: "I am CONCERNED," "I am UNCOMFORTABLE," and "This is a SAFETY ISSUE." Expect questions about when a nurse should escalate rather than wait.

Assertion steps. Remember the order: make an opening, state the concern, state the problem, offer a solution, reach agreement on next steps.

Conflict resolution. Know that DESC covers Describe, Express, Suggest, and Consequences, with consequences framed in terms of patient impact and team goals.

Formative feedback. Know the seven qualities and that feedback should focus on behaviors, not personal attributes.

Team event tools. Match brief (before the start), huddle (ad hoc, monitoring the plan), and debrief (after, reviewing performance) to the correct scenario.

Barriers. Recognize barriers such as hierarchy, defensiveness, lack of role clarity, and misinterpretation of cues as causes of communication failure.

Diagnostic safety data. Know that communication breakdowns appear in about one-third of diagnostic-related malpractice cases, and that outpatient settings account for the largest share of diagnostic-related communication failures in this module's data (**55%**).